

Initial management of acute upper GI bleeding (AUGIB)

For adults experiencing haematemesis, melaena or coffee-ground vomit

NB: DO NOT prescribe PPI prior to endoscopy

DO NOT prescribe TXA (ineffective in GI bleeding)

Disclaimer: This is a clinical template; clinicians should always use judgment when managing individual patients

Re-approved by ED guidelines committee on 1st Apr/26
Review due Jan27 . Trust Ref C188/2016

Patient details

Full name

DoB

S number

(use addressograph if available)

① Resuscitation bundle checklist

- ☐ Manage in safe area (in ED: resuscitation room)
- ☐ Involve senior doctor **NOW**
- ☐ Establish IV access - ideally x2
- ☐ Take blood for FBC, VBG, U&E, LFT, INR, clotting screen and G&S
- ☐ Call blood bank to cross-match 6 units of RBC
- ☐ Give appropriate crystalloid bolus
- ☐ Monitor vital signs / NEWS every 15min
- ☐ Monitor fluid balance
- ☐ Maintain Hb using restrictive transfusion strategy
 - ☐ If Hb below 80, transfuse (target Hb 70-100)
 - ☐ If Hb above 80, do not transfuse
- ☐ Reverse anticoagulation where relevant
 - ☐ If any of the below, reverse anticoagulation using the applicable UHL guidance
 - ☐ On Warfarin AND INR >1.5
 - ☐ On LMWH - contact on-call haematologist
 - ☐ On DOAC (a xaban or dabigatran)
 - ☐ If none of the above, no action required
- ☐ Restore haemostatic competence if needed
 - ☐ If any of the below, contact on-call haematologist:
 - ☐ Platelets <50 and likely still bleeding
 - ☐ INR >1.5
 - ☐ APTR >1.5
 - ☐ Fibrinogen <1g/L
 - ☐ If all four normal, no correction needed
- ☐ After resuscitation, repeat FBC and VBG

② Variceal bleed suspected?

- ☐ **YES**, as at least one of the below
- ☐ Previous variceal bleed / known varices
 - ☐ Known cirrhosis
 - ☐ Clinically jaundiced
 - ☐ Clinical ascites
 - ☐ Spider naevi / liver palms
 - ☐ Splenomegaly
 - ☐ Platelets <100 (unless known other cause)
- ☐ **NO**, as none of the above

③ Variceal bleed bundle

To prescribe in NC, go to Emergency Medicine (ED) > Bleeding / reversal meds (ED) > Variceal bleed bundle

- ☐ Terlipressin 2mg IV QDS for 72h
 - ☐ (tick here if contraindicated)
- ☐ Antibiotic prophylaxis for 72h (switch to PO after 24h if possible)
 - ☐ If penicillin-allergic:
 - ☐ Ciprofloxacin 400mg IV BD
 - ☐ Otherwise: Co-amoxiclav 1.2G IV TDS

④ Glasgow-Blatchford Score

Tick applicable criteria & record final GBS

- Urea (tick one box if raised)
- | | | |
|-------|---|--------------------------|
| > 6.5 | 2 | ☐ |
| > 8.0 | 3 | ☐ |
| >10 | 4 | ☐ |
| >25 | 6 | ☐ |
- Haemoglobin (tick one box if low)
- | | | |
|------------------------|---|--------------------------|
| <130 in male patient | 1 | ☐ |
| <120 in female patient | 1 | ☐ |
| <120 in male patient | 3 | ☐ |
| <100 | 6 | ☐ |
- Systolic BP (tick one box if low)
- | | | |
|------------------|---|--------------------------|
| <110 at any time | 1 | ☐ |
| <100 at any time | 2 | ☐ |
| < 90 at any time | 3 | ☐ |
- Heart rate >99 at any time
- | | | |
|---|---|--------------------------|
| Melaena (spontaneous or on DRE) | 1 | ☐ |
| Presenting with syncope | 2 | ☐ |
| Heart failure (known history, clinical signs or on ECHO) | 2 | ☐ |
| Liver disease (known history, or clinical or laboratory features) | 2 | ☐ |

- ☐ **12-23 High risk**
- ☐ **2-11 Medium risk**
- ☐ **0-1 Low risk**

⑤ GI team referral dataset

- Age, sex, NEWS and vital signs
- Whether or not variceal bleed suspected
- Whether or not patient takes an anticoagulant
- Any ReSPECT form details / ceiling of care
- Cognitive function & clinical frailty score (CFS)
- Glasgow-Blatchford Score (see box 4)
- Hb, acid base state, coagulation screen
- Interventions so far (blood, crystalloid etc.)

- IV access
- Take blood for FBC, VBG, U&E, LFT and coag
- Ask blood bank to cross-match 2 units of RBC
- If anticoagulated, pause or reverse as indicated
- Transfuse if Hb <70 (or <80 if acute coronary syndrome present). Transfuse only a single unit at a time then recheck FBC; target Hb 70-100.

* Contacting GI team

The team should be contacted by an ST3+ or consultant only

Ensure you can relay all the information stated in the referral dataset (see box 5)

- During hours:
 - GI 'registrar' on **07950 856 607**
 - If there is no timely response, escalate to on-call GI bleed consultant through UHL switchboard

- Out-of-hours:
 - On-call GI bleed consultant via switchboard

No inpatient OGD needed

From ED:
Discharge home
Check boxes 6 and 7 for indications for outpatient OGD and use template text from box 8 for your GP discharge letter
For inpatients:
Continue usual care

From ED:
Admit to AMU/AFU (ACB if variceal bleed)
For inpatients:
Monitor overnight
Refer to GI team mane

NB: Patients who deteriorate while awaiting OGD on a non-ITU ward **MUST** be escalated rapidly to the anaesthetic and GI teams unless agreed end-of-life care plan in place

From ED:
Admit to ACB unless ceiling of care below ACB
For inpatients:
Monitor closely or move to ACB if appropriate

Recommendations regarding patients' regular medications

- Continue low-dose (i.e. 75mg daily) aspirin
- Pause any other NSAIDs (including COX-2 inhibitors), DOACs and warfarin
- Pause any P2Y12 inhibitor-type antiplatelets unless patient has coronary artery stents, in which case follow the below:
 - If PCI >6 months ago, pause P2Y12 inhibitor
 - If PCI 6 months ago or less, omit until discussed with consultant interventional cardiologist (7/7, during working hours)

Completed by

Print name

Signature

Role

Date

Time

⑥ Indication for '2-week wait' referral for OGD? *tick any applicable criteria*

- ☐ Patients aged 55 years or older with unexplained and persistent recent-onset dyspepsia
- ☐ Patients of any age with dyspepsia who present with one or more of the below:
 - ☐ Chronic gastrointestinal bleeding
 - ☐ Progressive dysphagia
 - ☐ Progressive unintentional weight loss
 - ☐ Persistent (excluding cyclical) vomiting
 - ☐ Iron deficiency anaemia
 - ☐ Epigastric mass
- ☐ Patients presenting with any of the following, even in the absence of dyspepsia:
 - ☐ Dysphagia
 - ☐ Unexplained upper abdominal pain and weight loss, with or without back pain
 - ☐ Upper abdominal mass

tick here ☐ if no indications identified

OGD is urgent; inform patient and complete a 2WW referral on ICE:

New referral > Service Referrals (from tabs at the top) > Endoscopy (from tabs at the left) > Tick Gastroscopy OP (from the Outpatients menu column) > Click OK in the dialog box > Select Urgent (within 2 weeks) in the next dialog box then click OK > Complete referral form (in the Aims of procedure section, select Diagnostic from the menu)

In your discharge letter (see box 8 below), use the 'Scenario 1' paragraph

⑦ Might outpatient OGD referral need to be considered by GP? *tick any applicable criteria*

- ☐ Iron deficiency anaemia
- ☐ Unexplained weight loss
- ☐ Persistent vomiting and weight loss in the absence of dyspepsia
- ☐ Worsening of their dyspepsia known to have any of the following risk factors:
 - ☐ Barrett's oesophagus
 - ☐ Dysplasia
 - ☐ Atrophic gastritis (pernicious anaemia)
 - ☐ Intestinal metaplasia
 - ☐ Peptic ulcer surgery more than 20 years ago

tick here ☐ if no indications identified

NB: Referral will be at GP's discretion; **DO NOT** mention it to pt in order not to raise false expectations.
In your discharge letter (see box 8 below), use the 'Scenario 2' paragraph.

⑧ Nervecentre discharge letter template text

copy & paste the full text below into the 'GP Notes' box, then add / delete [including all of the help text shown in brackets] as required

Dear Doctor,

Your patient attended our ED with an episode of acute upper gastrointestinal bleeding.

Risk stratification according to the Glasgow Blatchford Score (GBS) found them to be at low risk of death from this event, and not requiring specialist intervention. Since no additional need for hospital admission could be identified, we were able to discharge the patient from the ED.

[Delete two of the following three paragraphs as consistent with boxes 6 and 7 above]

[Scenario 1] Your patient met the following criteria for urgent oesophago-gastro-duodenoscopy (OGD): *[copy and paste the specific criteria identified in box 6]*. We have therefore already completed a 2WW outpatient referral for the procedure.

[Scenario 2] We have identified the following issue that might, at your discretion, warrant further review and investigation: *[copy and paste the specific criteria identified in box 7]*. This can be arranged via the UHL Gastroenterology Advice & Guidance service.

[Scenario 3] A referral for outpatient oesophago-gastro-duodenoscopy (OGD) will not be needed.

[NB: You may add further actions that might be needed at the GP surgery here, but consider that our primary care colleagues' ability to act within 3 weeks will usually be very limited]